SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Nivolumab and ipilimumab for pleural mesothelioma

First-line unresectable malignant pleural mesothelioma in adults. · NCCP 00792 · version 2 · 42-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This treatment aims to control pleural mesothelioma that cannot be removed with curative surgery, help symptoms and extend life. It is usually given without an expectation of cure. Mesothelioma starts in the lining around the lung and is different from non-small-cell lung cancer.

Schedule
Each cycle lasts 42 days: nivolumab and ipilimumab on Day 1, then nivolumab alone on Day 22. Nivolumab is every three weeks and ipilimumab every six weeks. This regimen contains no chemotherapy. Continue while appropriate until progression, unacceptable toxicity or up to 24 months without progression. Your team reviews the plan and may delay or stop treatment.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Nivolumab
IMMUNOTHERAPY

- ☐ Immune inflammation in multiple organs
- ☐ Infusion reactions
- ☐ Delayed effects after treatment stops

Ipilimumab
IMMUNOTHERAPY

- ☐ Immune bowel, liver, skin and hormone inflammation
- ☐ Other serious organ inflammation
- ☐ Infusion reactions

Discuss treatment aims, alternatives, expected benefit and uncertainty, short-term and lasting harms, and effects on fertility, contraception, pregnancy, work and daily life. Your priorities and questions belong in the decision.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00792 v2.

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